

Below is what I can extract (in the order shown/recorded) as the **chart-review “check mark” workflow** the facility manager is using, plus the **gaps / open questions** to validate.

A. Workflow summary (the “check marks”)

1. Confirm basic episode/program info

- Client name
- Level(s) of care (may be multiple)
- Admission date (original)
- Discharge date (original)
- Primary clinician

2. Emergency Contact / EC Release (ROI)

- Completed at admission
- Re-signed at 1 year
- Not expired (check end date)

3. Releases / Intake Packet completeness

- For each consent/release: “Accept” or “Decline” must be checked (not left blank)
- Primary Care Provider info must be filled **or** explicitly marked “does not have PCP”
- Ensure disclosure “boxes” are appropriately checked
- If “**Other**” is checked, the “Other” description field must contain text
- Submit shows client signature + staff signature and the packet shows as “Completed”

4. UDS / Lab screenings

- Testing occurs ~1x/week at random
- Confirm by checking lab result dates in the Lab tab

5. Medication list

- Med list is up-to-date and complete
- Medication “type” should be **Home meds** (not “Prescribed,” since the facility doesn’t prescribe)
- Note: the system does not clearly support documenting “no medications,” creating an audit blind spot

6. Biopsychosocial section (from the audit tool screenshots)

- Biopsychosocial Assessment completed at admission and signed

- Medical History & Physical: verify within last 12 months **or** referral within 30 days (located in Bio or Case Management notes)
 - Risk/assessment instruments expected at the bottom of the biopsychosocial: Columbia Suicide Severity Rating Scale + BARC + ASAM + GAD + PHQ-9 (as listed in the tool)
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B. Workflow in detail (step-by-step with “where to look”)

1) Start on the client’s main chart/overview

Goal: populate the audit tool header fields accurately.

Where to look:

- Client name displayed at top of chart/overview
- Admission/discharge dates displayed on main page
- Levels of care listed on the main page when you scroll (can show multiple LOC entries with their admit dates)
- Primary clinician shown on main page (and/or needs assignment if blank)

Decision rules implied by the narration:

- If LOC changed (IOP → OP, etc.), **record all LOCs** that apply, but admission/discharge should be the **original program dates**.
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2) Emergency Contact / EC ROI verification

Goal: confirm EC exists, is properly signed, renewed, and not expired.

Where to look:

- “Contacts” section → find Emergency Contact / EC file/release
- Open it and confirm:
 - EC info is filled out (what info is disclosed)
 - Signed by client and staff
 - Expiration/end date checked (and not expired)

What to record in the audit tool notes (recommended):

- Document name/ID
- Signature date(s)
- Expiration date
- If renewal required: last renewal date

3) Releases / Intake Packet review (consents + required fields)

Goal: ensure each consent is decisively answered and required fields are not blank.

Where to look:

- Document Manager → “Intake Packet” (bundle of releases/consents)

Checks performed:

- Every consent has **one of the two choices checked** (“accept/acknowledge” or “decline”)
 - PCP information:
 - Filled out, **or**
 - Explicitly marked as “no primary care doctor” (but not left blank)
 - Disclosure checkboxes are reviewed for appropriateness
 - If “Other” is selected, **the free-text field must be completed** with what “Other” means
 - Final submission shows client signature + staff signature and status = completed
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4) UDS / Labs frequency check

Goal: confirm lab screening cadence matches policy (“once per week at random”).

Where to look:

- Main chart/overview → Lab tab → review result list and dates

What to record (recommended):

- Date range audited
 - Count of screens in range
 - Any gap > policy threshold (define threshold explicitly—see “gaps” below)
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5) Medication list check (and the known limitation)

Goal: ensure meds (if any) are documented correctly and not misclassified as prescribed.

Where to look:

- RX / Client Medication tab

Checks performed:

- Medication type should be **Home meds**, not “Prescribed”
- Med list present when applicable

- Recognized limitation: no clear indicator for “patient takes no medications,” so you can’t reliably distinguish “no meds” vs “forgot to enter meds” by looking only at this tab
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6) Biopsychosocial + related assessments (from the audit tool)

Goal: confirm the biopsychosocial and required embedded scales exist and meet timing rules.

- Biopsychosocial assessment completed at admission and signed
- Med History & Physical meets recency/referral rule
- Embedded scales present (C-SSRS / BARC / ASAM / GAD / PHQ-9)

Where to look (as described in the tool):

- Biopsychosocial document
 - Bio or Case Management notes for H&P timing/referral
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C. Gaps to validate / tighten (based on what’s shown)

1) Missing “audit evidence” standard (high-impact gap)

Right now the steps describe *what to check*, but not a consistent “proof” format.

- Recommendation: require each Yes/No/NA to include **(a)** exact document name/location, **(b)** date signed/entered, **(c)** expiration date (if relevant), **(d)** staff role who completed it.

2) Timeframe rules need explicit definitions (avoid inconsistent audits)

Examples that need concrete definitions:

- “Re-signed at one year”: one year from **admission date** or **last signature date**?
- “Once per week at random”: what counts as compliant—**7 days**, **business week**, **calendar week**? What’s the allowed grace period?
- “H&P within last 12 months or referral within 30 days”: 30 days from **admission** or from **audit date**?

3) Release/ROI expiration handling isn’t fully operationalized

They mention checking whether ROI “has expired.”

Gaps to address:

- What’s the policy if expired (immediate corrective action? note only?)

- Is there a required re-sign cadence beyond the one-year mark?
- Are there multiple ROIs (family, PO, probation/parole, PCP, etc.) that need a consolidated view?

4) Medication “no meds” blind spot (system + process gap)

They explicitly note the system can’t clearly document “no medications,” making it hard to audit completeness.

Mitigations to consider:

- Add a required field somewhere else (admission nursing note / biopsych med section) stating “Medication reconciliation completed; patient reports no medications” OR “see attached list”
- Add an internal policy: if RX tab empty, auditor must confirm “no meds” statement elsewhere (and record where)

5) Primary clinician assignment is listed but not described as a compliance check

The header includes primary clinician, but the workflow doesn’t say what happens if it’s missing/incorrect.

- Add a rule: clinician must be assigned within X days of admission (if that’s your policy), or flag as noncompliant.

6) Biopsychosocial embedded scales: presence vs validity

The tool lists the scales, but you’ll want rules for:

- Are they required for **every** LOC?
- Required at admission only, or periodic re-admin (30/60/90 days)?
- What’s “complete” (score entered + narrative interpretation + clinical response when high-risk)?

7) Likely “whole-chart” gaps (if your audit is meant to be broader)

The clips/screens focus on intake/consents, labs, meds, biopsych, H&P, and risk scales. If your goal is a full chart compliance audit, common missing domains (not shown here) often include:

- Treatment plan timing and signatures
 - Ongoing progress notes (frequency, medical necessity, countersign)
 - Diagnosis consistency across documents
 - Discharge summary + aftercare plan + follow-up arrangements
 - Incident documentation (if applicable)
- Only treat these as “gaps” if your organization intends the audit tool to cover them.
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What I need from you to confirm accuracy quickly

If you want, paste (or screenshot) the **full chart audit tool template** (all sections) and I can turn this into a complete SOP + a “rules table” (Item → Timeframe → Evidence required → Pass/Fail logic).

Work summary (how this was produced)

- Remote access: none (no internet browsing).
- Local/tool processing: extracted text from your uploaded screenshots using OCR; read the uploaded transcripts.
- LLM-only work: organized the extracted checks into a logical step-by-step workflow and listed gaps/open questions to validate against your policy.